

Final Integration of Developmental Trauma

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AGENDA

Arrival

Assessment

8 Phase Map

Reevaluation

Demonstration Session & Discussion

Journaling

Q & A

Arc of a Session

Step 1 - Arrival

Step 2 - Assessment

Step 3 - 8 Phase Map

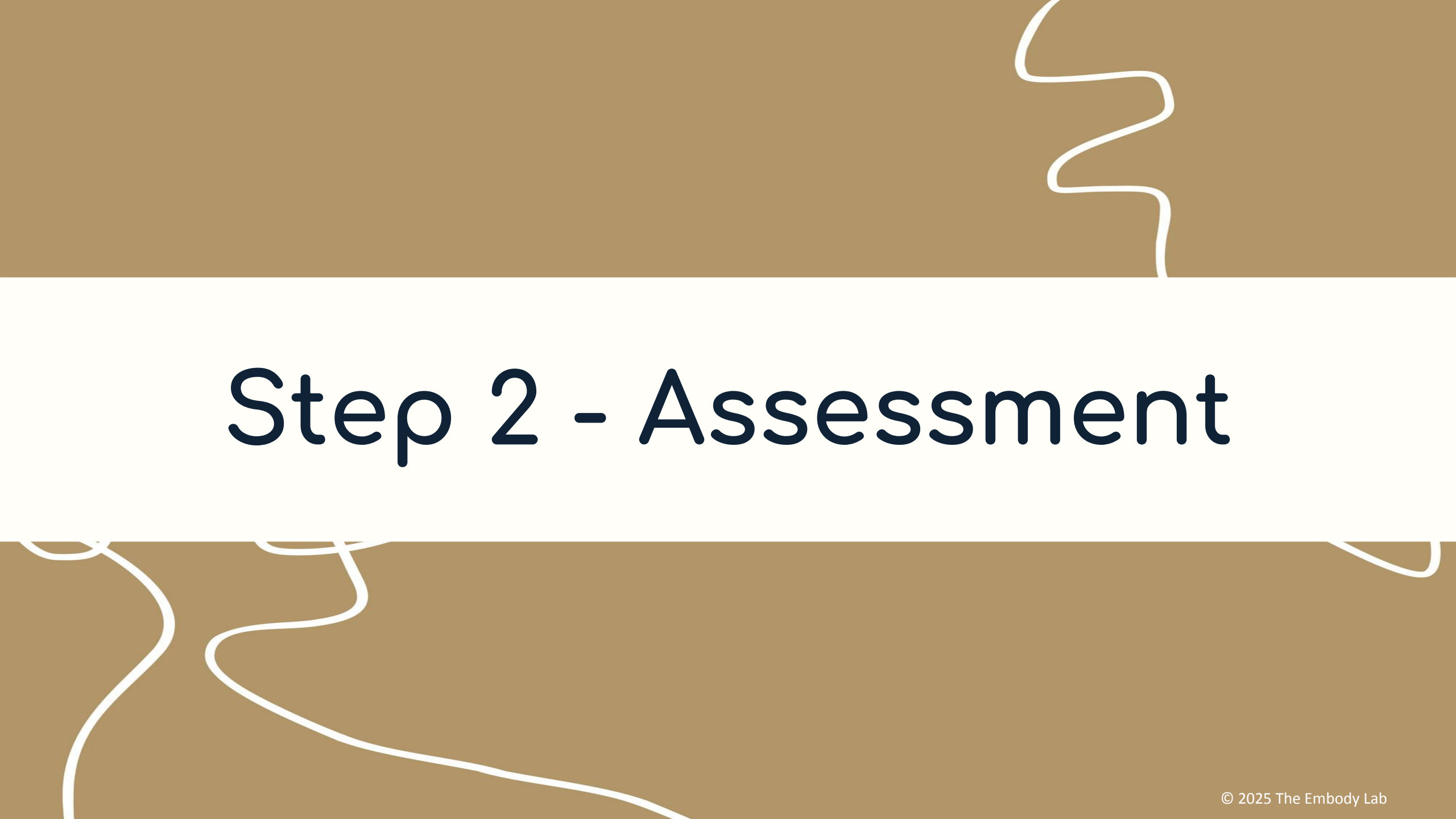
Step 4 - Reevaluation



Step 1 - Arrival

Arrival

- Orienting to the space
 - Ground
 - Attend to proximity
 - Find cues of “safe enough” in the here and now
- Invite the client to “check in”
 - What is on your mind?
 - What do you notice in your body?
 - What emotions are present?
 - Are there any intentions that you have for today’s session?



Step 2 - Assessment

Assessment

- What are the signs that we are in developmental trauma
 - See *Stages of Development Practitioner Worksheet*
 - See list of *Questions for Developmental Trauma*
- If client is showing signs of developmental trauma, what stage is present?
 - Pre- and Peri-natal Stage (0 - 6 months)
 - Infancy and Dependency Stage (6 months–1.5 years)
 - Independence in Relationship Stage (1.5 - 3 years)

Questions

- How would you describe your childhood?
- Who was in your family and who took care of you?
- What do you know of your mother's pregnancy?
- Do you know if you were wanted?
- What do you know about your birth?
- Were there any medical complications or early separations?
- What stories were you told about your childhood?
- How would you describe your mother?
- How would you describe your father?
- Did you have any other significant caregivers?
- How were your emotions (anger, fear, sadness, joy) responded to by family members?
- How did family members express their emotions?

STAGES OF DEVELOPMENT

Prenatal & Perinatal Stage (0-6 months)

Developmental Needs

- Safety in the womb and during birth
- Physiological support through caregiver regulation (nutrition, environment, attunement)
- Being emotionally welcome into life
- A protected entry into life where existence itself feels safe enough

Disruptions & Trauma Patterns

- Primordial/primal freeze: shutdown of brain, body, and developmental sequencing
- Prenatal stressors (maternal abuse, drug/alcohol exposure, lack of medical support, unsafe environment, unwanted pregnancy, high stressors for the mother during conception and pregnancy)
- Birth trauma (complications, C-section impact, invasive medical interventions, incubator time)
- Extended separation, neglect, or postpartum depression in caregiver
- Somatic presentation: frozen core, joint/muscle tension, infant dissociation, dysregulated nervous system, interrupted development, global mistrust

STAGES OF DEVELOPMENT

Prenatal & Perinatal Stage (0-6 months) continued...

Core Needs & Negative Cognitions

- Core need: It is safe for me to exist & I am welcome here, it's safe to be embodied
- Common NCs: "I shouldn't exist," "I am not wanted," "I cannot trust the world," "I do not belong," "my body is not safe."

STAGES OF DEVELOPMENT

Infancy & Dependence Stage (6 months-1.5 years)

Developmental Needs

- Safety & Security: protection, rest, reliable holding, consistent caregiving
- Connection: love, belonging, attachment, early communication, sense of - primary attachment
- Being tended to in a timely way when needs arise
- Validation & Empathy: being seen and felt, - foundational development of the self.
- Mirrored Play: spontaneous expression, joyful engagement, rhythm and co-regulation through play
- Agency: choosing and signaling needs
- Safety (Theme): establishing the body's baseline trust in protection and care in relation to the primary caregivers
- Emotional Regulation: co-regulation scaffolds the capacity for self-regulation later

STAGES OF DEVELOPMENT

Infancy & Dependence Stage (6 months-1.5 years) Continued...

Disruptions & Trauma Patterns

- Medical complications disrupting bonding and continuity of care
- Not supported in developmental needs such as crawling, pulling up, standing and walking, being pushed to walk, etc.
- Over-rigid caregiving (scheduled feeding, limited touch) or inconsistent attunement
- Somatic presentations: collapse in chest/shoulders, lack of spinal support, rigidity in arms/legs
- Psychological imprint: helplessness, rejection, unworthiness, loss of place, not in touch with core needs, not a good connection with core self, mistrust in the world as a safety net

Core Needs & Negative Cognitions

- Core needs at this stage: Safety, Connection, Validation, basic physical needs, Play, (limited) Agency, Emotional Regulation
- Common NCs: “I am not worthy of support,” “I cannot get my needs met,” “I cannot make it on my own,” “I am not good enough,” “It is not safe to rest,” “I will be abandoned”, “It’s not okay for me to have needs,” “When I am met, it is not what I truly need,” “My rhythm is not seen and matched”

STAGES OF DEVELOPMENT

Independence in Relationship Stage (1.5–3 years)

Developmental Needs

- Exploration and self-assertion while maintaining connection
- Autonomy without abandonment, protection without overprotection
- Embodied “No” as a foundation for agency and authenticity
- Creative exploration and becoming a scientist of the world

Disruptions & Trauma Patterns

- Shaming of needs or boundary-setting that is harsh or punitive
- Chronic power struggles, conditional love, neglect or abuse
- Overprotection (blocked autonomy) or premature independence (pushed out of dependency)
- Somatic presentations:
 - Fawning/Over-Adaptation: forward-leaning, collapsed chest, shallow breath
 - Chronic Defiance: rigid jaw, bracing shoulders, narrowed focus
 - Avoidant Independence: disconnected core, floating energy, lack of grounding
 - Collapse/Indecision: flaccid tone, unfocused eyes, low voice
 - Freeze in Choice: stillness without alertness, stiff extremities
 - Flight: excessive anxiety that is not regulated by caregiver, client stays stuck in anxious loops, high motor expressions such as running or fidgeting.

- Resource and pace sessions to prevent overwhelm

STAGES OF DEVELOPMENT

Independence in Relationship Stage (1.5–3 years) Continued...

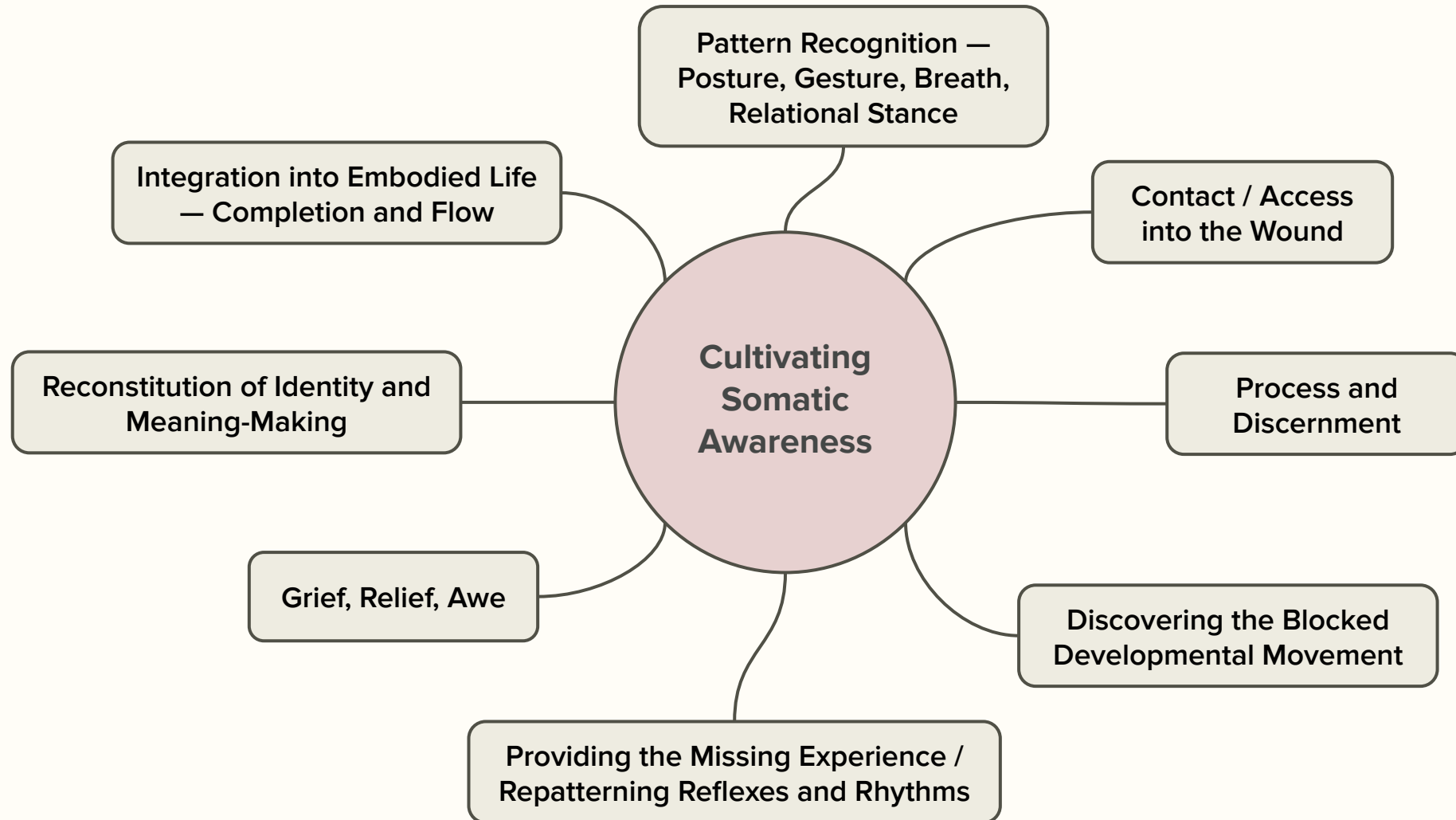
Core Needs & Negative Cognitions

- Core need: It is okay to be me, I can explore the world in safety, I can be in relation with the world and in support with my caregiver. I am powerful and independent and have preferences.
- Common NCs: “I must perform to be loved,” “It is not okay to be me,” “I cannot protect myself,” “I am unlovable”, “I am not supported in my creativity,” “I will be dominated when I exert needs,” “I can’t disagree or I lose connection with my caregiver,” “I have preferences that I have to resist to not disappear”



Step 3 - 8 Phase Map

The 8-Phase Map of Developmental Repair



HUB: Cultivating Somatic Awareness

ASSESSMENT

You are in this phase when:

- The client is beginning to notice sensation, posture, impulse, breath, or emotion
- There is increased contact with present-moment experience
- The practitioner is tracking somatic shifts as they arise
- Awareness expands or collapses depending on proximity to activation
- Client can stay in contact enough to remain curious

PRACTICES

As practitioner:

- Slow time down intentionally
- Orient the client to the present sensory field
- Name subtle shifts in sensation or breath
- Pendulate awareness between activation & resource
- Support interoceptive precision (micro-awareness)
- Ask brief, embodied questions:
 - *“What are you noticing now?”*
 - *“Where do you feel contact?”*
 - *“What happens as you stay with that?”*
- Anchor attention in the here-and-now body
- Invite shared tracking between both nervous systems
- Reinforce a stance of curiosity over meaning-making

1 - Pattern Recognition

(posture, gesture, breath, relational stance)

ASSESSMENT

You are in this phase when:

- Somatic, emotional, or relational sequences repeat
- Gestures or micro-movements recur without completing
- Similar relational responses emerge across topics
- Symptom clusters loop in predictable ways
- The cycle does not resolve or satisfy

PRACTICES

As practitioner:

- Gently mirror what you observe repeating
- Highlight patterns without pathologizing
- Enlarge subtle signals through amplification
- Help the client stay with the repeating impulse
- Explore how the body sustains the pattern
- Assess what is over-used and what is absent
- Support curiosity about the loop:
 - *“Let’s notice when this shows up again.”*

This phase is about **recognition, not resolution.**

2 - Contact / Access Into the Wound

ASSESSMENT

Signs you are entering access:

- There is a sudden intensification or collapse in experience
- Emotional expression emerges spontaneously
- The client is touched by the deeper impact
- They feel the weight or significance of the pattern
- Something old is being touched through the now

PRACTICES

As practitioner:

- Reduce stimulation and complexity
- Maintain grounded contact with the client
- Titrate entry (never force)
- Invite the smallest feelable piece
- Orient the client to safety in this moment
- Track breath, muscle tone, eyes, and orientation
- Help differentiate then vs now
- Repeatedly return to resourcing without abandoning the contact

3 - Process & Discernment

(the “soup” phase)

ASSESSMENT

Indicators:

- Confusion emerges
- Emotional and cognitive clarity dissolves
- Compensatory structures become visible
- The client struggles to know what they feel or want
- Things blur, conflict, or contradict

PRACTICES

As practitioner:

- Normalize confusion AS process
- Reinforce safety while in the unknown
- Support the body to bear ambiguity
- Bring in relational attunement
- Help identify what is known (even if small)
- Slow and soften internal urgency
- Separate fused experiences or meanings
- Introduce gentle counter-positions:
 - *“What if both could be true?”*
 - *“What happens if we pause in the not-knowing?”*

Do not rush clarity.
This is the digestion phase.

4 - Discovering the Blocked Developmental Movement

ASSESSMENT

Look for:

- Interrupted or incomplete movement
- Missing sequencing or flow
- Disorganized or fragmented movement
- Avoidance of certain spatial directions
- Difficulty with motor initiation
- Inability to embody push, reach, yield, pull, orient
- Difficulty choosing or committing
- Narrowing of movement possibility

PRACTICES

As practitioner:

- Observe where movement halts or collapses
- Follow the incomplete movement slowly
- Support micro-completions (tiny steps)
- Explore underlying impulse (push, pull, reach, turn, orient)
- Name what cannot yet occur
- Allow the body to experiment safely
- Offer just enough support or resistance for organization to emerge
- Help connect movement with meaning only afterward, not during

Focus on restoring **organizing sequences**, not performing them.

5 - Providing the Missing Experience / Repatterning Reflexes & Rhythms

ASSESSMENT

Indicators:

- A new developmental impulse awakens
- The body attempts something unfamiliar
- The client senses a possibility they never had
- Relational field shifts from reenactment to repair
- The system leans toward what should have happened originally

PRACTICES

As practitioner:

- Offer the experience that was absent:
 - *protection, attunement, support, encouragement, presence, boundary, softening, stillness*
- Allow the body to complete sequences (movement-based)
- Co-create new relational experiences
- Offer counter-shape when appropriate
- Reinforce agency AND safety
- Allow the client to feel received

TWO PATHS: Internally generated / Relationally generated

This is where **repair** unfolds.

6 - Grief, Relief, Awe

ASSESSMENT

Occurs when:

- Emotional release emerges spontaneously
- Deep tears, shaking, exhaling, softening arise
- Grief surfaces over what was lost
- Relief arises over receiving what was missing
- Awe emerges in the realization of possibility

PRACTICES

As practitioner:

- Witness with grounded presence
- Say LESS not more
- Validate grief without rescuing
- Allow time for the body to reorganize
- Slow movement and language significantly
- Reinforce permission to feel
- Maintain relational contact without agenda

Here the system **lets go**.

7 - Reconstitution of Identity & Meaning-Making

ASSESSMENT

Look for:

- Self-perception shifts
- New capacities recognized
- Narratives update
- More agency, less collapse
- Increased complexity & nuance
- Shame loses dominance

PRACTICES

As practitioner:

- Invite reflection on what has changed
- Help articulate new identity statements
- Affirm new internal organization
- Reflect back tangible differences
- Support imagining new relational possibilities
- Explore the “*who am I now?*”

This phase is not about idealization — it’s about **integration of truth**.

8 - Integration Into Embodied Life

(completion & flow)

ASSESSMENT

You see:

- New behavior in daily life
- New relational choices
- New boundaries enacted
- Faster recovery after activation
- Increased adaptability
- Emerging coherence

PRACTICES

As practitioner:

- Explore how the new capacity is showing up in life
- Identify where the client wants to bring this forward
- Rehearse real-world applications somatically
- Outline supportive structures
- Normalize setbacks
- Reinforce capacity rather than performance
- Help track flow and cyclic return

Integration is **lived**, not conceptual.



Step 4 - Reevaluation

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Markers of Growth and Observable changes

Can the client say:

- I'm able to understand my own responsibility or position in a relational dynamic.
- I'm able to see more of my own responses and patterns as opposed to just either taking it all on or blaming it all.
- I can recognize that age in the moment, I feel really little, vulnerable, fluttery.
- I can identify my own missing need in that moment.
- I verbalize my need.
- I can hold a clear boundary and negotiate my needs.
- I can appreciate and actually luxuriate in connection without that being a threat.
- I can pause.
- I can be responsive as opposed to reactive.
- I can notice my previous defense structures are less reflexive.
- I'm capable of being with discord or disagreement and staying connected to myself and other.
- I can yield in conflict.
- I can take action and follow my impulses without overwhelm.



Demonstration & Discussion



Journal Prompts

1. What are you most excited about?
2. What are the next steps for you?
3. What are you excited to bring into your work?
4. What are the unexpected little gems?



QUESTIONS